

THREE AUDIENCES WHO WILL NEVER AGREE ON WHAT A WEBSITE IS FOR

A hospital coordinator with an unfilled night shift, a travel nurse deciding where to register and a daughter working out whether her father can stay at home all arrive at the same domain. Cascadia Health is a human-led healthcare staffing company and care-support partner. We built the architecture that gives each of them a route of their own without fracturing a single brand.



ECONOMIES, ONE DOMAIN A facility is buying labour under time pressure. A clinician is selling labour and choosing where to invest their licence. A family is buying care for a person they love. A healthcare operator is buying strategic counsel. Four transactions, four vocabularies, four definitions of a good day, and one website trying to speak to all of them. Multi-service healthcare businesses usually fail in one of two ways. The site flattens everything into generic healthcare staffing solutions	FACILITIES Buying labour under time pressure	Coverage
	PROFESSIONALS Choosing where to invest a licence	Careers
	FAMILIES Buying care for someone they love	Care
	OPERATORS	

Multi-service healthcare businesses usually fail in one of two ways. The site flattens everything into generic healthcare staffing solutions language that converts nobody, or it bolts on a page per service until the navigation becomes a filing cabinet. Both come from organising around what the company sells instead of why each visitor came.

FOUR TRANSACTIONS ON ONE DOMAIN
FACILITIES Buying labour under time pressure Cover
PROFESSIONALS Choosing where to invest a licence Career
FAMILIES Buying care for someone they love Care
OPERATORS Buying strategic counsel Advice

THE CHALLENGE

THREE SIDES COMPETE FOR THE SAME HOMEPAGE

Healthcare staffing is a two-sided labour market with a consumer services business attached. Five problems shaped the build.

GAPS RUN ON THE CLOCK A coordinator with an unfilled night shift will not browse. If a request is more than one click away, they phone a competitor.	OPPOSITE WANTS Facilities want a deep, screened bench. Professionals want good assignments and a recruiter who answers. One page cannot persuade both.	TWO KINDS OF HIRING A per-diem shift is decided in minutes. A permanent placement is decided over weeks. One call to action cannot carry both.	SPECIALISMS MUST SHOW Behavioral health, memory care and correctional settings need named experience, or they are assumed absent.	DIFFERENT PROOF Facilities verify vendors. Clinicians check whether their data is safe. Families ask who is coming into the house.
--	--	--	---	--

The strategic question was not how to fit everything on the site. It was how early a visitor can stop reading things that are not for them.

THE SEARCHES BEHIND THE VISIT

EACH AUDIENCE SEARCHES IN ITS OWN WORDS

Healthcare staffing demand, clinician supply and permanent hiring are different intents. The architecture gives each its own page, so the pages do not compete for one head term.

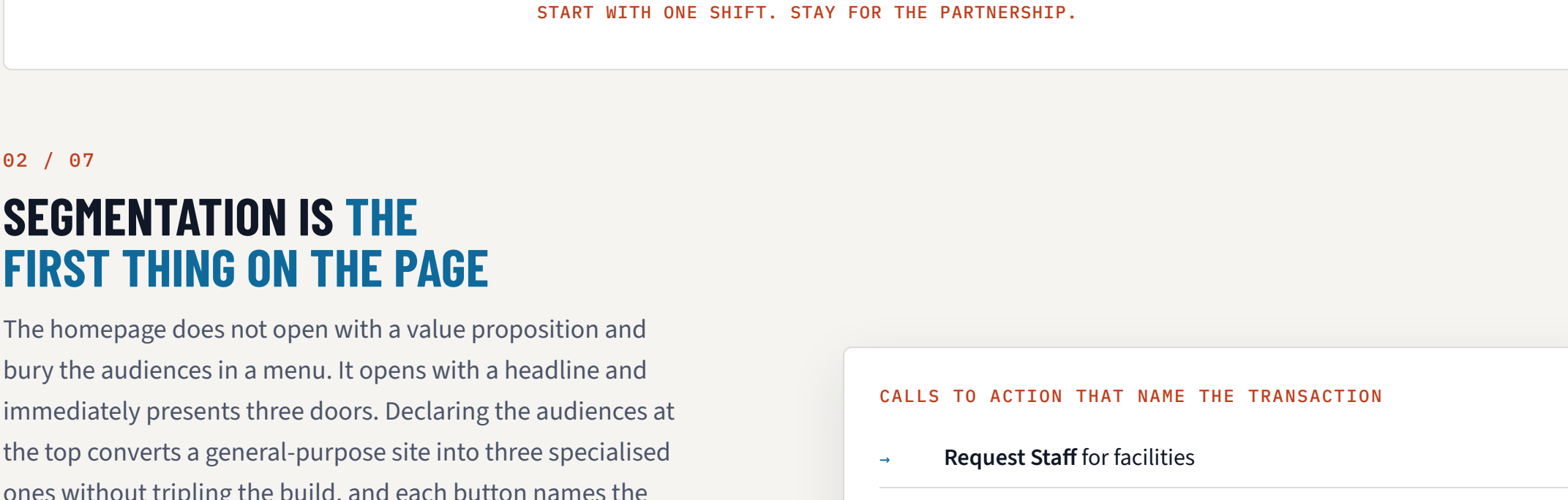
FACILITIES NEED COVER A coordinator looking for a healthcare staffing agency or medical staffing agency wants a request form in one click, and proof of a screened bench. Emergency and on-call coverage leads the page. Healthcare staffing agency Medical staffing agency Healthcare temp agency	CLINICIANS NEED THE RIGHT ROLE A nurse comparing a healthcare recruitment agency with a national job board needs the case for registering made before the form. The page argues first, then asks. Healthcare recruitment agency Medical recruitment agency	LEADERS NEED A LASTING ANSWER A structural vacancy that temporary cover only masks calls for direct hire and permanent placement, and for a partner who can plan beyond the next shift. Healthcare employment agency Healthcare staffing solutions Healthcare staffing company
---	--	---

WHAT WE BUILT

01 / 07

SIX SERVICES, ONE LADDER

The homepage names six medical staffing services: immediate and per-diem coverage, short and long-term assignments, direct hire and permanent placement, behavioral and mental health staffing, in-home family support and workforce consulting. Read in order, the first four and the last describe a deliberate progression. The facilities page says it outright: start with one shift, stay for the partnership.



02 / 07

SEGMENTATION IS THE FIRST THING ON THE PAGE

The homepage does not open with a value proposition and bury the audiences in a menu. It opens with a headline and immediately presents three doors. Declaring the audiences at the top converts a general-purpose site into three specialised ones without tripling the build, and each button names the transaction, so a click is already a qualified signal.

NO IRRELEVANCE	Every paragraph a visitor reads that was not written for them is a withdrawal from their patience.
COMMITTED COPY	Once a visitor has chosen, the language no longer has to hedge.
FOUR FUNNELS	Four outcomes can be tracked and improved independently.

CALLS TO ACTION THAT NAME THE TRANSACTION

Request Staff for facilities
Join the Talent Team for professionals
Request Family Support for families
Request Management Consulting Services for operators and investors, reached from navigation and footer

03 / 07

ONE TAXONOMY, THREE RESOLUTIONS

Care settings appear three times, at three levels of detail. The homepage list is sized for recognition: a visitor finds their own world in about three seconds. The facilities page adds environments that general agencies rarely name. The request form turns the taxonomy operational, because the difference between a nursing home and a skilled nursing facility decides who can be sent.

For a coordinator at a psychiatric hospital, seeing Psychiatric Hospital and Behavioral Health Facility as named, selectable options is the proof. The taxonomy is the credibility argument, delivered as structure.

SETTINGS NAMED AT THREE RESOLUTIONS

7 HOMEPAGE · RECOGNITION
10 FACILITIES · ADDS MEMORY CARE, CORRECTIONAL
19 REQUEST FORM · OPERATIONAL
Critical access hospitals VA hospitals Psychiatric hospitals Detention centers LTACHs Adult family homes

04 / 07

ELEVEN DISCIPLINES
Nurse Practitioners Licensed Practical Nurses Medical Assistants Mental Health Professionals Physical Therapists Respiratory Therapists Registered Nurses Certified Nursing Assistants Behavioral Health Technicians Social Workers Occupational Therapists
ASSIGNMENT TYPES
Per-diem PRN Contract Temp-to-hire Permanent

05 / 07

A RECRUITMENT PATHWAY THAT ARGUES BEFORE IT ASKS

Supply is the constraint in healthcare staffing, so every credible registration is inventory the business can sell. The professionals page works as a medical recruitment agency page should: it answers a known objection at each point. A recruiter who knows you answers the complaint of being a line in a database, and the full range of assignment types welcomes both the occasional per-diem worker and the permanent hire.

SAME LIST, BOTH SIDES	The same eleven disciplines appear to facilities and clinicians. A facility sees its bench, and a clinician sees themselves on it.
TRAVEL EARLY	Travel radius and relocation willingness sit in step one, because geography invalidates a match fastest.

06 / 07

ONE INTAKE PATTERN, THREE DIFFERENT FIRST QUESTIONS

Many sites segment at the hero and then collapse everyone into a shared contact form. Here each pathway ends in its own intake at its own URL. All three run seven steps with an identical progress indicator, and each opens with the lightest possible ask. That shared pattern is what makes four businesses feel like one product, and it turns a wall of fields into a staircase.

FACILITIES	Who and where: facility name, type and address, then the coordinator.
PROFESSIONALS	Name, location and how far they will travel.
FAMILIES	A name, a relationship and how they prefer to be contacted.

STEP ONE OF SEVEN, IN EVERY INTAKE

STEP 1 OF 7	14%
FAC	Facility name, type, address, coordinator
PRO	Name, location, travel distance, relocation
FAM	Name, relationship, contact method, best time

07 / 07

THREE AUDIENCES, THREE DIFFERENT PROOFS

Trust is not one thing here. Each audience is asked to risk something different, so each is reassured differently. A security note sits inside every form, at the point of hesitation rather than in the footer, and a repeated commitment that a person will answer addresses the common complaint of being processed by a portal and never called back.

FACILITIES	Screened before they're sent, credential vetting named, disciplines and settings listed, and a published process.
PROFESSIONALS	Documents reviewed by a real person, and a recruiter who knows them.
FAMILIES	Screened and verified, confidential by default, with personality counted in matching.

RESULTS & IMPACT

A WORKING ARCHITECTURE, STATED AT THE LEVEL IT CAN BE DEFENDED

No performance metrics were supplied for this engagement, and none are claimed. What follows is the delivered capability.

4 paths	Clear audience journeys Three care pathways plus an advisory practice, each with its own entry page, argument and conversion action.	6 services	Organised service architecture Six service lines arranged as a commercial progression from urgent shift to workforce partnership.
3 intakes	Purpose-built inquiry capture Three seven-step intakes sharing one pattern, each collecting what its audience can actually answer.	Recruit	A focused recruitment pathway A supply-side funnel with its own objection handling, process explanation and credential registration.
Named	Demonstrated clinical breadth Eleven disciplines and up to nineteen facility types named explicitly rather than claimed generally.	Scalable	A structure that extends New services, settings or audiences extend the existing pattern rather than requiring a rebuild.

No staffing volumes, placements, fill rates, response times, client names, revenue or conversion figures are claimed. None were supplied. Statements about screening and data handling on the client's own site are the client's representations.

WHAT'S NEXT

THREE ADDITIONS THAT WOULD MAKE IT SEARCHABLE BY CITY

Healthcare staffing search is overwhelmingly local, and both labour-market audiences ask well-defined questions before they transact. The next steps build on structure that already exists.

LOCATION PAGES
A page for each metro served, carrying the relevant settings and disciplines, turns a stated service area into a searchable one.
A CONTENT LAYER
What per-diem rates look like in a region, how credentialing timelines work, when agency cover is cheaper than overtime. Behavioral health, memory care and correctional staffing are the natural starting points.
INSTRUMENTATION
Completion by step for each intake, enquiry volume by pathway, facility-type distribution and registration-to-placement ratio.

QUESTIONS

ABOUT THIS HEALTHCARE STAFFING AGENCY CASE STUDY

WHAT DOES A HEALTHCARE STAFFING COMPANY LIKE CASCADIA OFFER? +

HOW DOES CASCADIA WORK AS A HEALTHCARE RECRUITMENT AGENCY FOR CLINICIANS? +

HOW DOES A FACILITY REQUEST STAFF? +

DOES YOUR BUSINESS SERVE AUDIENCES THAT WANT COMPLETELY DIFFERENT THINGS? +

START A CONVERSATION

DOWNLOAD THE FULL CASE STUDY